

Registration / Demographics

Patient Name:	Jenna M. Summers	MRN:	23456789
DOB:	March 5, 1987	Age:	36
Sex:	Female	Insurance:	Secure Health Plan
Address:	456 Maple Lane, Wellness City, ZZ 12345	Phone:	(555) 234-5678
Emergency Contact:	Alex Summers, (555) 345-6789	Account:	78901234
Encounter Date:	September 14, 2023	Check-In Time:	08:15 AM
Location:	Wellness City Endoscopy Suite	Attending Gastroenterologist:	Dr. David Harmon, MD
Referring Provider:	Dr. Lindsay Everett, MD	Chief Complaint/ Indication:	Dyspepsia
PMH:	Gastroesophageal Reflux Disease (GERD), Hyperlipidemia	PSH:	Appendectomy
FHX:	Father with colon cancer	SHX:	Nonsmoker, Social alcohol use
Allergies:	Penicillin	Problem List:	GERD, Dyspepsia
Current Medications:	Lansoprazole 30 mg daily, Simvastatin 20 mg nightly, Multivitamin daily, Vitamin D 1000 IU daily, Omeprazole 20 mg daily	Pre-Procedure Vitals:	BP: 118/78, HR: 75, SpO2: 98% on RA, ASA II

Informed consent was obtained from the patient, including a discussion of the procedure's risks, benefits, and alternatives.

Pre-Procedure Assessment & Anesthesia Record

Pre-Anesthesia Evaluation: Airway assessment revealed a Mallampati score of II. Cardiopulmonary examination was unremarkable. Patient confirmed NPO status since 10:00 PM the previous day, last clear fluids at 6:00 AM.

Planned Sedation: Moderate sedation planned using Midazolam 2 mg IV and Fentanyl 100 mcg IV. Anesthesia start time: 08:35 AM.

Anesthesia Team: Dr. Mark Lewis, MD (Anesthesiologist); Julie Carter, CRNA.

Procedure Times:

Time	Event
08:35 AM	Anesthesia start
08:45 AM	Procedure start (Scope in)
09:10 AM	Procedure end (Scope out)
09:15 AM	Anesthesia end
09:20 AM	Recovery start
10:00 AM	Recovery end, discharged home

Monitoring Trends:

Time	BP	HR	SpO2
08:35 AM	116/76	72	98%
08:45 AM	114/74	70	97%
09:00 AM	112/72	69	98%
09:10 AM	115/75	71	99%

Complications: None.

Post-Anesthesia Evaluation: Aldrete score of 10, patient discharged home with escort.

Procedure Note: Esophagogastroduodenoscopy (EGD)

Procedure: The patient was positioned in the left lateral decubitus position. A bite block was placed. Using a Olympus CF-H180AL endoscope, we carefully traversed the oropharynx and passed the upper esophageal sphincter.

Esophagus: The esophageal mucosa appeared normal. The Z-line was identified at 38 cm from the incisors. The gastroesophageal junction (GEJ) appeared normal without any evidence of rings, strictures, or esophagitis.

Stomach: Upon entering the stomach, the fundus, body, and antrum were examined. Notable for mild erythema in the antrum, which was consistent with mild gastritis. Normal rugal folds with adequate insufflation. The pylorus was easily traversed.

Duodenum: The first and second parts of the duodenum were examined. The villi appeared normal. No ulcers or significant inflammation were observed.

Retroflexion: Retroflexed view of the cardia and fundus appeared normal.

Findings:

- Mild erythema in the gastric antrum, suggestive of mild gastritis.

Impression: The findings are consistent with mild gastritis. No evidence of Barrett's esophagus or other significant pathology was noted.

Diagnosis: Mild gastritis.

Interventions: No biopsies were taken.

Endoscopy Team: Dr. David Harmon, MD (Endoscopist); Nurse Susan Lin, RN (Circulator); Robert Fields (Endoscopy Tech).

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Specimens/Lab/Pathology

No specimens were acquired during this procedure.

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After Visit Summary / Discharge Instructions

The patient awoke from sedation without any significant nausea or discomfort. She tolerated sips of water well and had stable vitals (BP: 120/78, HR: 74, SpO2: 98%).

Diet/Activity: Resume normal diet as tolerated. Avoid alcohol and heavy meals for the remainder of the day. Activities as tolerated, but avoid driving or operating heavy machinery for 24 hours.

Warning Signs: Seek medical attention for persistent vomiting, severe abdominal pain, fever, or difficulty swallowing.

Follow-Up: Return to clinic in 4 weeks for follow-up. Continue current medications and add PPI if symptoms worsen.

Medication Plan: Continue Lansoprazole as prescribed. No NSAIDs unless approved by provider.

Results Communication: Results of the examination and further recommendations will be communicated via phone or patient portal within 2 weeks.

The patient confirmed understanding of these instructions and was discharged in stable condition with her escort, Alex Summers.

RN Discharge Educator: Eliza Markson, RN, 10:05 AM.

Provider Signature: Dr. David Harmon, MD, Electronically signed on September 14, 2023, at 10:06 AM.

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